GAMSA · Gender-Affirming Medicines South Australia

Blood test guide: Spironolactone

Spironolactone (androgen blocker) · gamsa.au/monitoring

Key points Potassium and kidney tests, more often at first. Your oestradiol tests carry on as usual.

Applies to me	Test	When	Aim	Talk to your prescriber if
☐	**Potassium, sodium and kidney function (eGFR)**	Before starting, then at each review. The product leaflet suggests more often at first: weekly, then monthly for 3 months, then every 3 months for a year, then every 6 months	Within the lab's range Other guidance (Endocrine Society 2017): potassium every 3 months in the first year, then yearly	Potassium is flagged high, your kidney result drops, or you start a blood-pressure tablet (ACE inhibitor or ARB), an anti-inflammatory (NSAID), a potassium supplement or a salt substitute
☐	**Oestradiol and total testosterone**	At each review, as in the oestradiol table	Spironolactone may not bring testosterone below 2 nmol/L; its effect is judged by your body changes	The changes are not what you hoped for
☐	**Blood pressure**	At each review	Not too low for you	You feel dizzy or light-headed, especially when you stand up

A guide to what to expect, not a schedule to follow on your own. Your prescriber sets the timing.

Reading your results

- Lab reports mark results as high or low using the sex on your Medicare or lab record. On hormones, that range may not fit your body. In the first year your prescriber may ask the lab for both the male and the female ranges.
- A single flagged result is a reason to talk with your prescriber, not to change a dose on your own.

Things that can make a result look wrong

- Kidney function (eGFR) is worked out from your creatinine, age and recorded sex, so the change over time tells more than one result.

Sources: AusPATH 2026 pp27, 28, 31, 36, 53, Table 8 p37; Spiractin CMI and Aldactone PI s4.3-4.4; Equinox guide 2020 p12. Full references and detail for clinicians: gamsa.au/monitoring.

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